

GUIDELINES

2020 European guideline on the management of genital molluscum contagiosum

Abstract

Molluscum contagiosum is a benign viral epidermal infection associated with a high risk of transmission. This guideline focuses on sexually transmitted molluscum contagiosum. Diagnosis is clinical, based on characteristic lesions—called *mollusca*—which appear as dome-shaped, smooth-surfaced, pearly, firm, skin-colored, pink, yellow, or white papules, 2–5 mm in diameter, with central umbilication. Dermoscopy may aid in diagnosis.

Therapeutic options include physical treatments (e.g., cauterization, curettage, cryotherapy), topical chemical agents (e.g., podophyllotoxin, imiquimod), or expectant management with spontaneous resolution in immunocompetent individuals. In pregnancy, physical methods such as cryotherapy are considered safe. Immunosuppressed patients may develop severe, recalcitrant lesions requiring systemic or off-label treatments such as cidofovir, imiquimod, or interferon.

Patients diagnosed with molluscum contagiosum should be offered screening for other sexually transmitted infections.

Introduction and Methodology

Molluscum contagiosum is a benign viral infection of the epidermis associated with high transmissibility and increasing prevalence worldwide.

This guideline addresses genital, sexually transmitted molluscum contagiosum in adolescents (aged ≥ 16 years) and adults. Its primary objectives are to

provide evidence-based recommendations on diagnosis, treatment, and prevention strategies to reduce reinfection and transmission.

All authors contributed equally.

Declarations of interest:

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The guideline was developed by reviewing existing literature, including the 2014 British Association for Sexual Health and HIV (BASHH) guideline and the 2015 Centers for Disease Control and Prevention (CDC) recommendations. A comprehensive literature search was conducted covering publications from 1980 to January 2019 (see Appendix 1: Search Strategy). Evidence was graded according to the "European STI Guidelines: Protocol for Production and Revision (April 2020)," using a modified GRADE system (see Appendix 2: Grading of Evidence).

The development process followed the standard operating procedures of the International Union Against Sexually Transmitted Infections (IUSTI) and the European Dermatology Forum (EDF), and was supported by the Dermato-Venereological Branch of the European Union of Medical Specialists (UEMS). Input was received from dermatovenereologists and members of the European Academy of Dermatology and Venereology (EADV), IUSTI, and EDF, and consensus was achieved through discussion.

The guideline adheres to the IUSTI methodology: <https://iusti.org/wp-content/uploads/2020/04/ProtocolForProduction2020.pdf>

Aetiology and Transmission

Aetiology

Molluscum contagiosum is caused by a large double-stranded DNA virus belonging to the *Poxviridae* family, genus *Molluscipox*.

Two main subtypes—MCV-1 and MCV-2—account for nearly all clinical cases, although rare subtypes have been identified through genotypic analysis. MCV-1 is the most prevalent globally, though distribution varies by region.

Two distinct clinical presentations are recognized:

1. Lesions on the face, neck, trunk, and arms—predominantly seen in children.
2. Genital, pubic, lower abdominal, upper thigh, and/or buttock lesions—commonly associated with sexual transmission and typically observed in young adults.